

Paediatric Outpatient Empiric Antibiotic Guidelines 2026

Indication	First Line Therapy	Second Line Therapy (If failure of first line or for other clinical considerations)	Typical duration of effective antibiotics (to be guided by clinical response)
Respiratory Tract Infections			
Community Acquired Pneumonia	Amoxicillin 60-90 mg/kg/day in 3 divided doses (max 1 g/dose)	Cefuroxime 20-30 mg/kg/day in 2 divided doses (max 500 mg/dose) (if non anaphylactic allergy) <u>Penicillin Allergy:</u> Either Cefuroxime as above <u>OR</u> Clarithromycin 15 mg/kg/day in 2 divided doses for 7-10 days (max 500 mg/dose)	5-7 days for beta-lactams **Dual cover with beta-lactam and macrolide highly discouraged for outpatients. Consider discussion with Paeds ID.
Atypical Pneumonia	Clarithromycin 15 mg/kg/day in 2 divided doses for 7-10 days (max 500 mg/dose)	Azithromycin 10 mg/kg/day OD for 3 days	Per agent
Aspiration Pneumonia	Augmentin 30-50 mg/kg/day in 2 divided doses <u>Severe:</u> Augmentin 67.5 mg/kg/day in 3 divided doses (max: susp 12.5 ml/dose and tab 625 mg/dose)	<u>Penicillin Allergy:</u> Clindamycin 30 mg/kg/day in 3 or 4 divided doses (max 450 mg/dose)	7 days
Ear, Nose and Throat Infections			
Otitis Externa	Ofloxacin 0.3% Ear Drop (Tarivid Otic); 6m-12y: 2 drops BD and ≥12y: 5 drops BD <u>OR</u> Ciprobay HC Ear Drop 3 drops BD (≥1y)		7 days

Severe Otitis Externa with spreading cellulitis beyond ear canal	Ciprofloxacin 20-30 mg/kg/day in 2 divided doses (max 500 mg/dose)		7 to 10 days
Acute Otitis Media	Amoxicillin 90 mg/kg/day in 2 or 3 divided doses (max 1 g/dose)	Cefuroxime 20-30 mg/kg/day in 2 divided doses (max 500 mg/dose) (if non anaphylactic allergy) <u>Penicillin Allergy:</u> Azithromycin single dose of 10 mg/kg <u>OR</u> Clarithromycin 15 mg/kg/day in 2 divided doses	<2 years: 10 days 2 to <6 years: 7 days ≥ 6 years: 5 to 7 days
Acute bacterial sinusitis	Augmentin 67.5 mg/kg/day in 3 divided doses (max: susp 12.5 ml/dose and tab 625 mg/dose) + Amoxicillin 30 mg/kg/day in 3 divided doses to make up 90 mg/kg/day of Amoxicillin component	<u>Penicillin Allergy:</u> Trimethoprim-sulfamethoxazole 8-12 mg/kg/day of TMP in 2 divided doses (max 160 mg TMP/dose) + Clindamycin 30 mg/kg/day in 3 or 4 divided doses (max 450 mg/dose)	10 days, to continue 7 days beyond symptom resolution
Bacterial lymphadenitis	Augmentin 67.5 mg/kg/day in 3 divided doses (max: susp 12.5 ml/dose and tab 625 mg/dose)	<u>Penicillin Allergy:</u> Clindamycin 30 mg/kg/day in 3 or 4 divided doses (max 450 mg/dose)	10 to 14 days
Bacterial tonsillitis/pharyngitis	Amoxicillin 50 mg/kg/day OD or in 2 divided doses (max 1 g/ day)	Augmentin 30-50 mg/kg/day in 2 divided doses <u>Penicillin allergy:</u> Clindamycin 30 mg/kg/day in 3 divided doses (max 300 mg/dose)	10 days

Urinary Tract Infections (UTI)			
Cystitis	Augmentin 30-50 mg/kg/day in 2 divided doses (max: susp 12.5 ml/dose and tab 625 mg/dose)	<u>Penicillin allergy:</u> Cefuroxime 10-15 mg/kg/dose in 2 divided doses (max 500 mg/dose) Please discuss with Paeds Renal or Paeds ID if considering above	5-7 days
Pyelonephritis/presumed upper tract UTI	Augmentin 30-50 mg/kg/day in 2 divided doses (max: susp 12.5 ml/dose and tab 625 mg/dose)	<u>Penicillin allergy:</u> Cefuroxime 10-15 mg/kg/dose in 2 divided doses (max 500 mg/dose) Please discuss with Paeds Renal or Paeds ID if considering above	10-14 days
Skin and Soft Tissue Infections			
Mild to Moderate Cellulitis	Cephalexin 60 mg/kg/day in 4 divided doses (max 500 mg/dose)		5-7 days if exposure to freshwater/seawater, add Ciprofloxacin 20-30 mg/kg/day in 2 divided doses (max 500 mg/dose) If exposure to sewage water, add Ciprofloxacin and Metronidazole 30 mg/kg/day in 3 divided doses If exposure to water bodies, duration of treatment may extend up to 14 days If suspect MRSA, consider sending cultures and use Linezolid or Bactrim

Bite Wounds	Augmentin 67.5 mg/kg/day in 3 divided doses (max: susp 12.5 ml/dose and tab 625 mg/dose)	<u>Penicillin Allergy:</u> Trimethoprim-sulfamethoxazole 8-12 mg/kg/day of TMP in 2 divided doses (max 160 mg TMP/dose) + Clindamycin 30 mg/kg/day in 3 or 4 divided doses (max 450 mg/dose)	5 days Macaque bites: PO Valacyclovir 60 mg/kg/day in 3 divided doses (max 1 g/dose) as post exposure prophylaxis for Herpes B virus Alternative is PO Acyclovir 100 mg/kg/day in 5 divided doses (max 800 mg/dose)
Scarlet fever	Amoxicillin 50 mg/kg/day OD or in 2 divided doses (max 1 g/ day)	<u>Penicillin allergy:</u> Clindamycin 30 mg/kg/day in 3 divided doses (max 300 mg/dose)	10 days
Eye Infections			
Periorbital Cellulitis	Augmentin 45-50 mg/kg/day in 2 divided doses (max: susp 12.5 ml/dose and tab 625 mg/dose)	<u>Penicillin Allergy:</u> Cefuroxime 20-30 mg/kg/day in 2 divided doses (max 500 mg/dose) (if non anaphylactic allergy)	7-10 days
Dental Infections			
Odontogenic infections	Augmentin 30-50 mg/kg/day in 2 divided doses (max: susp 12.5 ml/dose and tab 625 mg/dose)	<u>Penicillin Allergy:</u> Clindamycin 30 mg/kg/day in 3 or 4 divided doses (max 450 mg/dose)	5 days
Intra-Abdominal Infections			
Clostridium Difficile Diarrhoea	PO Metronidazole 25-30 mg/kg/day in 4 divided doses (max 500 mg/dose)	PO Vancomycin 40 mg/kg/day in 4 divided doses (max 125 mg/dose for mild disease, 500 mg/dose for severe disease)	10-14 days

Bacterial Gastroenteritis (GE) – antibiotic treatment not indicated unless in cases of: • Salmonella typhi (typhoid fever) • Non-typhoidal Salmonella (NTS) • Shigella to reduce symptom duration and transmission	• Severe traveller's diarrhoea – Azithromycin 10 mg/kg/day OD (max 500 mg) for 3 days pending results after sending stool culture, OCP and if any antibiotic exposure, C.diff toxin. • Uncomplicated/mild Salmonella typhi gastroenteritis – Azithromycin 10-20 mg/kg/day OD for 5-7 days OR Ciprofloxacin 30 mg/kg/day in 2 divided doses for 7-10 days; If bacteraemia or severe/complicated illness, parenteral antibiotics recommended at least for initial phase of treatment • Uncomplicated/mild Salmonella non-typhi gastroenteritis - consider PO Azithromycin (D1: 10 mg/kg OD, D2-5: 5 mg/kg OD) for all immunocompromised patients and infants <1yo. Evaluate immunocompromised and young infants for bacteraemia or severe/complicated illness, and if present, IV Ceftriaxone is recommended. Discuss with Paeds ID. • Shigella – Azithromycin 12 mg/kg/day OD on Day 1, 6 mg/kg/day OD on Day 2-5; Ciprofloxacin 20 mg/kg/day in 2 divided doses for 3-5 days. Cultures highly recommended in view of resistance profile. • Campylobacter – if severe illness or risk of it, then Azithromycin or Ciprofloxacin (dosing as above).
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